



Transfusion Policy Manual

Informed Consent for Transfusion

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Clinical Staff

Introduction

Patients must be fully informed of the risks, benefits, and possible alternatives to proposed blood or blood product treatment by having a discussion with their health professional. Patients should be fully involved in their care planning and the information should be provided in a format that is easy for the patient to understand. Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) is committed to optimising the patient journey by engaging patients and consumers in goal directed care.

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Policy Principles

The purpose of this policy is to provide guidance to healthcare professionals on the minimum mandatory requirements for obtaining informed consent for blood and blood product transfusion at SCGOPHCG. It should be read in conjunction with the [WA Health Department Consent to Treatment Policy](#).

Where a patient refuses blood products follow the [Refusal of Blood & Blood Products Policy](#), and a Refusal / Restricted Consent of Blood Products (Form D) must be completed.

Procedure

A medical officer (MO) must gain informed written consent from the patient, or have sighted a valid consent form, **BEFORE PRESCRIBING** any blood product.

This includes:

- Fresh Blood Products: red cells, fresh frozen plasma, platelets and cryoprecipitate
- Fractionated plasma products: immunoglobulin, albumin preparations and plasma derived factor concentrates

1. Consent Discussion

The following information **MUST** be discussed with the patient and/or next of kin (if patient unable to consent themselves):

- Ensure an accurate patient transfusion history is obtained and documented
- the reason for the proposed blood product transfusion
- the proposed, or potential, blood products for transfusion
- the risks and benefits of blood products, AND the risks or consequences of not receiving the products
- discuss the likelihood of transfusion reactions and/or adverse events
 - there is a small risk of adverse effects with any transfused blood product
 - most common transfusion reaction symptoms are fever, chills and urticaria
 - acute and delayed haemolytic transfusion reactions, anaphylaxis, transfusion associated circulatory overload (TACO) and transfusion-transmitted bacterial infection are some significant and potentially life-threatening complications of transfusion
- the availability and appropriateness of any other blood management strategies
- an opportunity for the patient/NOK to ask questions

A health service approved interpreter should be utilised where the patient and/or next of kin has limited proficiency in English.

Communication methods should be tailored to the patient and their needs. Written transfusion education materials should be supplied in appropriate format and language for the patient. Education materials are available via [Transfusion Services- Patient information and languages](#) page. The patient “Blood Transfusion Consent Information” is attached via perforation to the paper version of the Consent to Blood Products (Form C) and should be provided to the patient at time of consent if this form is utilised.

This information should be discussed and provided regardless of the consent form utilised for obtaining transfusion consent [[Consent to Treatment \(Form A\)](#), or [Consent to Blood Products \(Form C\)](#)].

2. Consent Forms

Consent to Blood Products (Form C) should be filed at the front of the Medical Record, behind Goals of Patient Care (if patient has one).

Transfusion consent can be obtained utilising either a [Consent to Blood Products \(Form C\)](#) (valid for the corresponding admission only unless the “recurrent transfusion” box is ticked) or, where transfusion may be required due to a surgical intervention or procedure, a [Consent to Treatment \(Form A\)](#) may be used if transfusion consent is discussed, and the YES box ticked at the time the consent to intervention or procedure was obtained.

Transfusion consent obtained via a Consent to Treatment (Form A), is valid for the whole admission during which the intervention or procedure that was consented to occurred, including the related inpatient recovery. *A Consent to Treatment Form A must NOT be used to consent for blood products where the patient is not undergoing a treatment or procedure, or for recurrent transfusion consent.*

Where transfusion of a blood product is unrelated to the intervention/procedure on Consent to Treatment (Form A), a Consent to Blood Products (Form C) should be completed.

Consent forms must legibly contain the following:

- **Patients' relevant diagnosis/condition/treatment for requiring transfusion**
- **Full name and signature of the Patient** OR "person responsible for giving consent if NOT patient"/ Next of Kin (NOK)
- **Medical officer's Full name and signature**
- **Medical officer's position**
- **Date consent signed**

An interpreters' declaration should also be completed where an interpreter was utilised to provide transfusion consent information.

Consent obtained via phone from "Person responsible for giving consent if NOT the patient"

Where consent is obtained from a person responsible for giving consent who is NOT the patient, (i.e. the NOK) via phone call, the full name of the person with whom consent was discussed, and their relationship to the patient, MUST be documented on the consent form and notated as "phone consent obtained" and dated. This process should also be documented in the patient's medical record.

Once the patient is stable and has capacity, they should be the one to provide informed written consent for any ongoing transfusion requirements.

3. Recurrent/Chronic Transfusion Consent

For patients requiring recurrent transfusion to manage chronic disease, consent is valid for **one year** from date of consent if the "Recurrent transfusion" box is ticked on the Consent to Blood Products (Form C), unless the condition changes or consent is withdrawn.

This should NOT be completed during an emergency transfusion event. Where a patient is unable, due to long-term illness/disability, to provide signed consent for recurrent transfusion, this should be discussed with the NOK and the consent to blood products (form C) completed and signed by MO and NOK.

At time of [Consent Discussion](#), the medical officer must also clearly convey that:

- The transfusion consent form will be valid for 12 months
- Consent can be withdrawn at anytime

At each transfusion event where the "recurrent transfusion" consent form is valid, clinical staff should verbally confirm patients' ongoing consent to transfusion.

4. Emergency/ Inability to Consent

As per the [WA Health Consent to Treatment policy 2023](#):

"Treatment can only be provided without consent where necessary to save a person's life, prevent serious injury to the person's health or prevent the patient from suffering severe pain or distress in circumstances where:

- the patient is incapable of giving consent
- the patient does not have an Advance Health Directive applicable and available
- it is not practical to determine whether the patient has a substitute decision maker who can be readily identified and immediately available.

Treatment without consent must be:

- reasonably required to meet the emergency
- in the patient's best interests
- the least restrictive of the patient's future choices.

The rationale for treatment without consent must be clearly documented in the patient's medical record. The medical record must state details of attempts made to contact the substitute decision maker."²

The substitute decision maker (SDM) is that as defined by the [Guardianship and Administration Act 1990](#) Section 110 ZJ: *Order of priority of persons who may make treatment decision in relation to patient (p.101)* and 110 ZD: *Circumstances in which person responsible may make treatment decision (p. 95)*. This is also depicted in the [WA Hierarchy of treatment decision-makers](#).

This section (below) located on page two of the Blood Product Transfusion Form should be completed in an emergency where consent is unable to be obtained

To be completed if unable to obtain consent		
This patient was unable to consent to transfusion of blood products because the transfusion was urgent/emergency AND the patient's conscious state was impaired due to ☐ illness/injury ☐ sedation in ICU ☐ anaesthesia. There is no evidence that the patient objects or would have objected to receiving a blood product transfusion e.g. refusal of treatment form, advanced health directive.		
Signature of Doctor _____	Designation _____	Date _____

Consent for Blood Products should be sought from SDM, or patient where possible, as soon as practicable post the emergency care event if further blood product transfusions are required.

Compliance, monitoring and evaluation

Non-adherence to this policy may result in adverse outcomes for the organisation and/or the patient due to poor, or ill-informed, consent practices. The SCGOPHCG Blood Management Committee is responsible for ensuring these policy requirements are met, and monitors compliance through regular auditing processes.

Related Documents

Legislation:

- Guardianship and Administration Act 1990
- Human Tissue and Transplant Act 1982

Authorising Policy and Standard/s:

- NSQHS Standard 7 Blood Management
- NSQHS Standard 2 Partnering with Consumers
- [WA Health Consent to Treatment Policy](#)

Standards, procedures, guidelines:

- [SCGOPHCG Administration and Checking of Blood Products](#)
- [Australian and New Zealand Society of Blood Transfusion and the Australian College of Nursing guidelines for the Administration of Blood Products](#)

Supporting documents

- [Consent to Blood Products \(Form C\)](#)

References

1. Australian and New Zealand society of blood transfusion, Australian College of Nursing. Guidelines for administration of blood products (Internet). Sydney: Australian & New Zealand Society of Blood Transfusion Ltd; 2019 Oct (cited 2023 Mar 21). Available from :<https://anzsbt.org.au/wp-content/uploads/2020/03/ANZSBT-Administration-Guidelines-Revised-3rd->

[edition-Publication-Version-FINAL- 20191002.pdf](#)

2. Government of Western Australia Department of Health. Consent to treatment policy (Internet). Perth: Government of Western Australia; 2023 Mar (cited 2023 Mar 14). Available from: <https://www.health.wa.gov.au/About-us/Policy-frameworks/Clinical-Governance-Safety-and-Quality/Mandatory-requirements/Consent-to-Treatment-Policy>

Authorisation & Version Control

Policy Sponsor	Medical Co-Director Cancer, Imaging and Clinical Services			
Policy Contact	CNC Transfusion			
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Standards Applicable	NSQHS Standards (Version 2): 			

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.